

TFP EXTENDED: DEVELOPMENT AND RECENT ADVANCES

John F. Clarkin, Eve Caligor, and Julia Sowislo

Abstract: Recent advances in the understanding of personality pathology have contributed to an emphasis on the core of personality pathology as deficits in self-functioning and interpersonal functioning at different levels of severity that must be assessed for clinical intervention. In concert with these conceptual and empirical advances, transference-focused psychotherapy (TFP), an empirically supported psychodynamic treatment for borderline personality disorder, has been in constant development with extensive clinical use. This article describes an object relations model for conceptualizing and assessing levels of personality organization, a transdiagnostic approach to personality pathology, and related treatment modifications, thus expanding the utilization of TFP beyond borderline personality disorder to the full range of personality dysfunction. The core of this treatment approach is a sequential interpretive process between patient and therapist. This process takes place within the context of a structured treatment frame tailored to the unique individual with problems in self-functioning and interpersonal functioning in his/her particular environment.

Keywords: object relations theory, personality, personality disorder, psychodynamic treatment of personality disorder, transference-focused psychotherapy

Transference-focused psychotherapy (TFP) is an empirically supported (Clarkin et al., 2001; Clarkin, Levy, Lenzenweger, & Kernberg, 2007; Doering et al., 2010) individual psychotherapy based upon psychodynamic object relations theory (Kernberg, 1984). TFP was initially developed for the intensive treatment of patients with the categorical

John F. Clarkin, Ph.D., Clinical Professor of Psychology in Psychiatry, Department of Psychiatry, New York Presbyterian Hospital—Weill Medical College of Cornell University, White Plains, New York.

Eve Caligor, M.D., Clinical Professor of Psychiatry, Clinical Professor of Psychology in Psychiatry, Columbia University Vagelos College of Physicians and Surgeons, New York.

Julia F. Sowislo, Ph.D., Instructor of Psychology in Psychiatry, Department of Psychiatry, New York Presbyterian Hospital—Weill Medical College of Cornell University, White Plains, New York.

diagnosis of borderline personality disorder (Clarkin, Yeomans, & Kernberg, 1999). Recent advances in the understanding of personality disorder and disturbed personality functioning have enabled us to extend the clinical application of TFP to patients suffering from deficits in self-functioning and interpersonal functioning at different levels of severity. The range of severity spans from, at the mildest level of severity, neurotic level of personality organization, and with increasing levels of severity, high-borderline organization, mid-borderline organization and, finally, at the level of most extreme severity, low-borderline level of organization (Caligor, Kernberg, Clarkin, & Yeomans, 2018). The enhanced application of TFP is called TFP extended (TFP-E).

In this article we provide an overview of recent developments of three central aspects of the TFP-E approach: (1) a model of personality pathology (object relations theory), (2) the assessment of levels of severity of personality organization, and (3) the basic principles of TFP-E as applied to the individual patient with a particular level of personality pathology severity. Whereas other articles in this series discuss the application of TFP-E to various patient groups and treatment settings, this overview focuses on the individual treatment format for adults with various levels of severity of personality organization.

RECENT ADVANCES IN THE PSYCHOTHERAPY FIELD

Advances in TFP have occurred in a context of advances in the wider psychotherapy field. Whereas the randomized clinical trial continues to be dominant in assessing the impact of a particular intervention, it is now commonly recognized that the randomized clinical trial based on the selection of patients on the basis of individual *Diagnostic and Statistical Manual of Mental Disorders (DSM)* diagnoses, including individual personality disorder diagnoses, has serious limitations (Norcross & Wampold, 2018). More precisely, within the criteria-defined *DSM* categorical system, multiple comorbid conditions and varying levels of severity assure substantial patient heterogeneity that influences treatment outcome. Future research must capture this patient complexity as related to treatment outcome.

In contrast, the *DSM-5* alternative model for personality disorders (AMPD; American Psychiatric Association, 2013) attempts to address the issue of heterogeneity and severity by defining the core of personality pathology as dysfunction in self functioning and other functioning (Clarkin, Caligor, & Sowislo, 2020) and construction of a severity rating for each patient. This approach is in line with conceptualizations of personality pathology as a large general factor of dysfunction (Sharp

et al., 2015), similar to our construct of borderline organization (Kernberg, 1984). Some have gone a step further in addressing heterogeneity, noting that every patient is unique, and that treatment must be personalized and address the individual in his/her environmental context (Wright & Woods, 2020).

The manualized treatment approach with the goal of encoding clear and consistent guidelines and procedures for the treatment of every diagnostic condition is not only daunting but redundant given the multiple comorbidities among patients. Instead, Barlow, Allen, and Chocate (2004) have suggested a transdiagnostic approach in which there are common therapeutic approaches to domains of functioning such as the various manifestations of anxiety. To further refine the match between the patient and the particular treatment approach, precision medicine has been applied to psychotherapy with attempts to match patients to specific treatments when multiple approaches are available (DeRubeis et al., 2014). This approach would use patient characteristics beyond the diagnosis per se to guide treatment selection (Keefe et al., 2020). Others (Norcross & Lambert, 2018) have summarized the work of successive review groups on empirically based psychotherapy relationships; moving beyond the examination of specific treatment techniques, they underscore the empirically supported importance of the therapist–patient relationship to treatment outcome.

Advances in psychoanalytic thinking about personality functioning and dysfunction have also contributed to the overall development of TFP-E. The development and articulation of the *Psychodynamic Diagnostic Manual* (PDM-2; Lingardi & McWilliams, 2017) is an example of contemporary developments in psychodynamic approaches to the description and assessment of personality pathology. Where TFP-E remains somewhat discrepant with much of the larger psychoanalytic world is the TFP-E model's emphasis on patient assessment, definition of treatment goals, and active pursuit of transference themes in the patient's everyday life and with the therapist.

These advances in the overall world of psychotherapy development combined with our own theoretical, clinical, and research experience have resulted in major refinements in our assessment and treatment of the heterogeneity of patients who meet criteria for personality disorders. We highlight the following: Our clinical assessment of the patient is consistent with the *DSM 5*'s AMPD but focuses specifically on the level of personality organization, diagnosed using the clinical Structural Interview or a semistructured interview, the Structured Interview of Personality Organization—Revised (Clarkin, Caligor, Stern, & Kernberg, 2016). Our treatment manuals are principle-driven, rather than describing procedures in lockstep fashion, with the aim of assisting the

clinician in applying the principles flexibly to the individual patient in different clinical situations and phases of treatment. The degree to which the treatment is structured from the beginning varies with the level of personality organization, as does the progression of levels of interpretation of the dominant object relations (Clarkin et al., 2020).

Our extensive clinical and research experience with the assessment and treatment of a wide range of patients with personality pathology has led to the following two-faceted formulation: (1) the complexity of the individual patient goes beyond the assessment of traits and categorical diagnoses of personality disorder, and (2) the treatment of these complex individuals must be flexible and adapted to the level of personality organization and environmental condition of the patient. This interaction of patient complexity/severity and treatment flexibility is captured in our current conceptualization of levels of personality organization as matched to the process of therapeutic interpretation within a carefully constructed treatment frame (Clarkin, Caligor, & Sowislo, in press). In the next sections, we describe the theoretical model, the nosology and assessment, the treatment, and related clinical research. Recent modifications, refinements, and advances in TFP are woven into this description.

OBJECT RELATIONS MODEL OF PERSONALITY PATHOLOGY

The model of personality pathology and levels of personality organization is based upon the clinical and theoretical writings of Otto Kernberg (Kernberg, 1975, 1984), along with recent reformulations with colleagues and related empirical work (for a summary see Horz-Sagstetter et al., 2018). This model predates and has been a contributor to Criterion A of personality pathology in the recently published *DSM-5 AMPD* (American Psychiatric Association, 2013).

The concept of identity and its healthy or maladaptive development is the cornerstone of the object relations theory model of personality functioning and disorder. Object relations theory refers to a group of psychodynamic models that view the internalization of early patterns of relating as central to psychic development and function.¹ Early and continuing interactions between the developing infant, child, and adolescent contribute to accumulating mental representations of self and others that are affectively charged. Assisted by a supportive interpersonal environment with a predominance of positive affect, the individual with normal identity formation possesses a core sense of self that is relatively stable, coherent, and realistic across different situations. This central concept of self enables the individual to experience a cognitive-affective

connection with others that is nuanced and well modulated. In contrast, failures in normal identity formation can lead to various levels of identity pathology, described as high-, mid-, and low-level borderline organization (Clarkin et al., 2020). These failures of normal identity formation are characterized by a sense of self and others that is superficial, unstable, and often distorted, accompanied by affects that are predominantly negative and poorly modulated. Failure of identity formation reflects the impact of splitting-based defenses, which dissociate mental states and representations of self in relation to other colored by positive affect from those colored by negative affect states, interfering with normal integrative processes and with identity consolidation.

NOSOLOGY: LEVELS OF PERSONALITY ORGANIZATION

The nosology that flows from this model combines dimensions of severity of pathology of internal object relations as described, with tendencies related to affiliation (introversion vs. extraversion), while providing a prototypic classification of personality pathology across four levels of personality organization. The four levels of personality organization are characterized by different levels of identity integration, quality of object relations, defensive functioning, management of aggression, and moral values. Determination of level of personality organization has direct clinical implications with regard to prognosis, treatment planning, and clinical process. This object relations model of levels of personality organization has influenced the *DSM-5 AMPD* (American Psychiatric Association, 2013). This dimensional model describes personality disorders along two dimensions, namely level of personality functioning (Criterion A) and pathological personality traits (Criterion B).

Specifically, the levels of personality organization have a strong similarity to the levels of personality functioning in Criterion A of the AMPD (see Table 1). We provide examples of the four prototypes below.

Prototypic Patient at Neurotic Organization

Mr. N* is a 35-year-old married lawyer with two young children. He was seen in consultation with complaints of anxiety, “problems with work,” and problems with self-esteem. Mr. N described his work as a

*All names and details have been changed to protect patient privacy.

TABLE 1. Relationship of Levels of Personality Organization and Levels of Personality Functioning Scale, Alternative DSM-5 Model for Personality Disorders Criterion A

	AMPD Level 1	AMPD Level 2	AMPD Level 3	AMPD Level 4
Neurotic Personality Organization	Consolidated identity; deep relationships, some with conflict; normal modulation of aggression and moral functioning	Some difficulty with self-esteem; conflicted about goals; cooperation with others conflicted		
High-Level Borderline Organization	Identity diffusion; some difficulty with goals; some deep relations but with conflict; repressive defenses; variable moral functioning	Vulnerable self-esteem; goals are often a means of gaining others' approval; excessively self-referential; cooperates with others for personal gain		
Mid-Level Borderline Organization	Identity diffusion; difficulty with personal goals; need fulfilling relations with others; splitting defenses; variable moral functioning and control of aggression		Ability to understand others limited; capacity for positive and enduring connections with others significantly impaired	
Low-Level Borderline Organization	Severe identity diffusion; exploitative relations; absence of guilt; potential for aggression against others			Weak or distorted self-image; unrealistic or incoherent personal goals; pronounced inability to understand others; relations with others limited by profound disinterest or expectation of harm

junior partner at a competitive law firm as challenging, engaging, and intellectually fulfilling. Nevertheless, he often felt anxious and inadequate in the workplace, and had received feedback that a tendency to be "too detail-oriented" was starting to slow his advancement in the firm. In the interview, Mr. N was emotionally constricted and extremely reserved, but nevertheless conveyed a deeply felt attachment to his wife and young children and he provided a lively, three-dimensional picture of his loved ones. He understood that his wife, and to some degree his children as well, experienced him as emotionally distant, and though deeply troubled by this, he found himself unable to change his behavior. Though free of sexual symptoms, he reported limited sexual interest. He had a small group of close friends dating back to high school and college days but was by his own account not especially social. After hearing Mr. N's story, the consultant found himself impressed by Mr. N's conscientiousness and by his motivation for treatment.

Case Formulation. Mr. N represents the structural features of the neurotic level of personality organization. He benefits from a stable and realistic sense of self and others and is able to pursue and invest in long-term goals, expressions of fully consolidated identity. Though emotionally constricted, he has a capacity for in-depth object relations and is able to empathize with the internal experience of his loved ones. Higher level defenses predominate, while the impact of repression-based defenses introduce rigidity and inhibition in the domain of affective expression. Aggression is well modulated and moral functioning is intact.

Prototypic Patient at High Borderline Organization

Ms. H, a 45-year-old, divorced nurse practitioner without children was seen in consultation with the complaint of chronic low mood. She did not endorse tearfulness or neurovegetative symptoms of depression but described a lifelong history of self-criticism and feelings of inferiority and inadequacy; "I see myself as a waste of space, I don't live up to my potential, but maybe I have no potential." At the same time, in her professional life, Ms. H was regarded as an effective, compassionate, and dedicated clinician. When confronted with the discrepancy between how she was viewed at work and how little she thought of herself, Ms. H was quietly dismissive, explaining that she was able to derive only limited pride or support to her self-esteem from her work and was instead preoccupied ("tormented") with failed aspirations, abandoned decades earlier, to become a physician. Ms. H. described a

series of failed 1- to 3-year-long relationships, “serial monogamy,” with men whom she initially loved and felt passionate toward but ultimately found disappointing. She oscillated between viewing this repeated experience as an expression of her own psychology and viewing it as her partners over time proving not to be whom they initially seemed to be. The interviewer noted that Ms. H’s descriptions of the men in her life were somewhat superficial, two-dimensional, and vague, while descriptions of her friends and colleagues were more specific. She maintained positive relationships with her work colleagues, men and women, and had a small circle of longstanding female friends. Despite her self-deprecation, Ms. H was appealing and engaging. The consultant found it easy to imagine that she was an asset to the clinical team, and also that she did not have difficulty generating interest from men.

Case Formulation. Ms. H illustrates many core features of high-level borderline personality organization. She demonstrates mild identity disturbance, with superficiality and instability in her experience of others and a pervasively negative coloration of her sense of self. While able to successfully pursue a career, her capacity to emotionally invest in and obtain satisfaction from her accomplishments is to some degree limited. Ms. H is able to maintain stable, if somewhat superficial relationships, with friends and colleagues. She presents with a combination of higher level and splitting-based defenses, with idealization/devaluation coloring and disrupting her romantic life. She has no antisocial features, and pathology of aggression is largely limited to attacks on the self and devaluation of her partners.

Prototypic Patient at Mid-Level Borderline Organization

Ms. B, a 28-year-old, single woman working part time as a receptionist, presented with a chief complaint of “problems in my love life.” Ms. B described how upon meeting a man she would feel, “This is the one who will solve all my problems,” only to find herself within weeks enraged and frustrated. She explained that when her feelings toward her boyfriends changed, she found it difficult to control her anger at them; on one occasion, she had verbally assaulted and physically threatened a man whom she had been dating until he called the police, though she had no history of physical abuse. On other occasions she threatened to harm herself when men did not behave as she felt they should. Ms. B had few friends—“more acquaintances, actually”—who came and went. She had been estranged from her parents since they discovered she had on one occasion taken “just twenty dollars” (“I was

desperate") from her mother's purse; when the interviewer asked if Ms. B could empathize with her parents' feelings, Ms. B responded, "I know it looks bad, but you'd think they could give me a break!" She otherwise denied a history of stealing or of any other illegal activity, though she routinely spent time on the job doing online shopping and making social calls, and she frequently called in sick; she justified these behaviors on the basis of the boring and routine nature of her work. She had held a series of short-lived jobs, which she described as dreary, boring, and pointless, though she could not think of anything else that she would like to do. Ms. B described feeling unhappy and resentful much of the time; she enjoyed shopping but otherwise had few interests. She felt her life was "going nowhere," and she was plagued by chronic feelings of emptiness. The consultant found himself feeling burdened by the profound and pervasive emptiness that characterized Ms. B's narrative and experience, along with the chaotic level of her functioning in multiple domains.

Case Formulation. Ms. B illustrates many core features of mid-level borderline personality organization. Identity disturbance is manifested in an unstable and superficial sense of self and others, an inability to invest in professional goals or personal interests, and painful feelings of meaninglessness and emptiness. Object relations are superficial and based on need-fulfillment, and splitting-based defenses predominate, accounting for the superficial, polarized, and unstable quality of her experience. Aggression is poorly integrated and poorly modulated, expressed in hateful temper outbursts and efforts to control significant others in a hostile way. She demonstrates significant, but not extreme, pathology of moral functioning; her antisocial traits are limited, and she is able to empathize with moral standards ("I know it looks bad, but . . ."), even while failing to change her behavior.

Prototypic Patient at Low-Level Borderline Organization

Mr. L, a 38-year-old, married, unemployed self-described "businessman," without children, was seen in consultation requesting stimulants "for my ADHD." Mr. L explained that challenges in relation to time management and organization had made it difficult to excel in his work, and he had benefited from stimulants in the past. As the interview proceeded, it emerged that since graduating college Mr. L had been let go from a series of jobs for hostile and combative behavior. Most recently, he had been fired after physically threatening a co-worker and then pushing him against a copy machine, an act he described as

“fully justified under the circumstances,” because the co-worker was not showing Mr. L the kind of deference that he insisted his greater level of experience and intelligence demanded. Mr. L had a history of abusing expense accounts in his jobs, and a criminal record for attempting to steal office equipment from a previous employer. Mr. L had lost sexual interest in his wife early in the marriage, and he routinely visited prostitutes, engaging in practices involving threatening and humiliating his partners without penetration. He explained that he stayed in his marriage because his wife owned the apartment they lived in, and he enjoyed living off her income. In the interview, though initially smooth-talking, his underlying hostility and contempt soon became overt, as did his derisive, challenging attitude toward the interviewer. Toward the end of the evaluation the interviewer explained he would need to see formal testing before prescribing a stimulant, but in the interim they could discuss other treatment options for ADHD. At that point Mr. L abruptly stood up, cursed the interviewer for deceiving him and wasting his time, announced, “There’s no way you’ll see a dime from me!” and stormed out of the office. The interviewer was left feeling shaken, but also disdainful of Mr. L.

Case Formulation. Mr. L illustrates poorly integrated and poorly modulated aggression in the setting of extreme pathology of object relations and deficits in moral functioning, the hallmarks of low-level borderline personality organization. Mr. L’s identity is poorly consolidated, with superficial representations of self and others and an inability to pursue goals or to invest in professional or personal interests. Object relations are based on ruthless exploitation and intimidation of others. Mr. L’s personality functioning and psychological life are organized largely in relation to expressions of poorly integrated and poorly modulated aggression in the form of violent outbursts and rage, cruelty, sadism, hatred, and devaluation. He engages in antisocial behavior—including lawlessness—and typical ethical standards are not only disregarded, but even viewed with contempt.

TRANSFERENCE-FOCUSED PSYCHOTHERAPY— EXTENDED TREATMENT MODEL

The TFP-E treatment model can be articulated at a general level on the basis of its treatment focus, that of promoting identity integration, and its intervention strategies, which combine structure and limit-setting with psychodynamic exploration. Recent advances have elaborated how, within the framework of these general principles, strategies

of intervention must be differentially applied according to patient severity and phase of treatment. Our extensive clinical and research experience with narcissistic personality organization and borderline personality organization patients has made it clear that patients do not enter treatment with the same capacity for change. Patients at different levels of personality organization experience crucial differences in their perception of self and others and their level of affect regulation in daily interactions, informing their experience of the treatment setting and of the therapist. In addition, it is our clinical observation that patients begin treatment in different phases of their severe personality disorder. The natural history of borderline personality disorder involves more impulsivity, affect dysregulation, and self-destructive behavior in the 20s and less self-destructive behavior but more depression and continued social dysfunction in the 30s and 40s. These factors require sophisticated initial assessment and individual case formulation.

Model of Change

The overall aim of TFP is to reduce patients' symptoms and interpersonal difficulties manifested in work/profession and friendship/intimacy deficits. This aim is accomplished through focus on the patient-therapist relationship, which exposes and provides opportunity to modify the mental representations of self and others that drive the patient's experience and maladaptive behavior. Because patients at a neurotic level of organization and those at the borderline level of organization differ in terms of cognitive biases and affective dysregulation, the TFP-E treatment approaches to these two groups of patients vary accordingly.

For patients at the neurotic level of personality organization, transference manifestations are subtle, the distortions in self-other representations are nuanced, and affect regulation is generally evident but variable in conflict situations. In contrast, for patients at a borderline level of organization, transference manifestations are intense and dominated by primitive elements, self-other representations are characterized by sharp division of positive and negative evaluations, and affective reactions are extreme. From a psychoanalytic object relations view, the predominance of highly charged negative affect states is seen as driving the activation of splitting-based defenses and related dissociative processes. In particular, negative affective experience invades and colors representations of self and others, and these representations are dissociated from positive affective experience. These dynamics interfere

with identity consolidation while introducing instability and conflict into interpersonal relations.

Consistent with the dominance of negative affect in the object relations model of personality pathology, the treatment model posits a central role for affect activation and increasing affect containment in the therapeutic process (Caligor, Diamond, Yeomans, & Kernberg, 2009; Levy et al., 2006). This model acknowledges the necessity of affect arousal in the treatment setting, made safe by the containing structure provided by the treatment frame and contract. The structured treatment setting provides opportunity to examine, clarify, confront, and modify extreme mental representations and related affects activated in the emotionally “hot” experience of others. This approach is consistent with current understanding of primitive affects and their contribution to numerous forms of psychopathology (Panksepp & Biven, 2012). The hypothesized mechanism of change in TFP is increased affect regulation achieved through the growing ability of the patient to reflect upon and to contextualize highly charged mental states and experiences of self in relation to others. Momentarily activated, split states can now be reflected upon and contained within a larger context that is better integrated and more benign and offers awareness of negative consequences of impulsive action.

Patient Assessment and Treatment Selection

The goal of the initial clinical assessment of an individual with suspected personality pathology is to understand current symptoms and difficulties, to diagnose the level of personality organization, to provide feedback to the patient, and to negotiate a treatment plan.

The assessment of levels of personality organization by using the clinical Structural Interview (Kernberg, 1981) or the semistructured interview Structured Interview of Personality Organization—Revised (STIPO-R; Clarkin et al., 2016; Stern et al., 2010) enables the clinician to diagnose the patient’s level of personality organization, essential for treatment formulation and determination of the parameters treatment. Empirical support for this typology was found by using an advanced latent structure model known as finite mixture modeling to identify subgroups of patients with borderline personality disorder (Lenzenweger, Clarkin, Yeomans, Kernberg, & Levy, 2008). Three subgroups were identified, characterized by different combinations of paranoid orientation to others, aggressive attitudes and behavior, and antisocial behaviors traits. These results have since been replicated (Hallquist & Pilkonis, 2012; Yun, Stern, Lenzenweger, & Tiersky,

2013), suggesting that the subtypes may be important to guide treatment approaches.

TFP is typically applied at a frequency of two sessions per week over a treatment duration of at least 1 year, depending on the treatment goals and severity of personality organization. However, since public health systems in many countries provide funding for only one session per week, there are ongoing clinical efforts to adapt TFP to a once-weekly frame.

Principal Intervention Strategies and Techniques

Key features of the TFP-E treatment model include: (1) framing the treatment with a verbal contract, (2) an interventional stance characterized by concerned technical neutrality on the part of the therapist, (3) focus on disturbed interpersonal behaviors both in the patients' relationship with the therapist and in extra-therapy current relationships, (4) use of a sequential process of interpretation to modify the patients' mental representations of self and others, leading to (5) changes in the patients' current interpersonal world of work and intimate/love relations (see Table 2).

Treatment Frame. Following a thorough assessment, the TFP therapist shares the diagnostic impression, focusing on identity and personality functioning and linking this to the patient's presenting complaint. Next, patient and therapist jointly identify treatment goals, focusing on improving concrete aspects of self and interpersonal functioning. Specific goals function as the "north star" of the treatment and help to avoid separation of treatment from the patient's day-to-day functioning. Before the therapy formally begins, the TFP therapist articulates a verbal treatment agreement, or "treatment contract," with the prospective patient in which the responsibilities of both therapist and patient are stated, and a discussion of each participant's responsibilities evolves. The treatment agreement establishes the treatment frame along with the necessary conditions for treatment, while providing a degree of structure needed for effective treatment of individuals with personality pathology. The degree of structure required in the treatment is directly related to the severity of personality pathology, with more severe pathology requiring a more elaborate and highly structured treatment agreement designed to contain potential threats to the patient and treatment due to the patient's behavioral dysregulation (acting out). The patient is responsible for coming to the sessions on time and discussing as freely as possible what is on his/

TABLE 2. Levels of Personality Organization and Treatment Planning

	Neurotic Personality Organization	High-Level Borderline Organization	Mid-Level Borderline Organization	Low-Level Borderline Organization
Clinical Objectives	Increase flexible functioning in areas of conflict	Greater depth and stability in experience of self and others	Resolution of destructive behavior; greater depth in experience of self and others	Increase of behavior control; modulation of aggression
Prognosis	Excellent	Good	Fair	Guarded
Treatment Frame	Less need for structured contract setting and monitoring	Explicitly agreed upon treatment contract promotes productive clinical process	Carefully constructed treatment contract is essential	Contracting must be extensive; focus on secondary gain and safety of patient and therapist
Technical Neutrality	Easy to maintain	Provides avenue to interpretation	Usually maintained; must abandon in times of crisis	Difficult to maintain due to behavioral dyscontrol
Interpretive Process	Use of entire process of interpretation; often focused on extra transference material	Use of steps in interpretation on transference and extra transference material	Much use of clarification and confrontation needed to proceed to later steps in interpretive process	Focus on keeping the treatment frame and liberal use of clarification and confrontation
Common Transference Patterns	Develops gradually and is affectively well modulated; realistic quality and relative stability over time	Early idealized transferences can keep paranoid object relations out of treatment	Develops rapidly, is affectively charged and extreme; confusing and chaotic quality with role reversals and abrupt shifts; early transferences predominantly paranoid	Affectively charged; predominantly paranoid
Common Countertransference Patterns	Relatively well integrated in form of thoughts or associations; therapists affected in subtle and socially appropriate ways which may be almost imperceptible at first	Subtler than in mid/low borderline personality organization; therapist feels less controlled and has more inner space to reflect	May be extremely uncomfortable, difficult to contain, experienced as imposed on; therapist feels controlled and driven to action	Extremely uncomfortable; therapist feels controlled and driven to action

her mind. Procedures for managing any self-destructive or suicidal behavior are discussed. The patient is responsible for engagement in school or work during the treatment period, whether it be part-time volunteer work or full-time paid engagement. This is less relevant for neurotic and most high-level borderline personality organization patients who function in occupational life, but most important for poorly functioning mid- and low-level borderline personality organization patients. The therapist is committed to listening intently to the feelings, thoughts, and concerns of the patient and to commenting when deemed helpful. By structuring the treatment with explicit roles and behaviors for therapist and patient, both effective interaction and deviations from it can be identified. It is only after the treatment frame has been articulated that one can identify and interpret deviations from the frame, which presupposes a patient motivated to overcome symptoms and difficulties with the assistance of a therapist who is not omniscient but helpful.

Technical Neutrality. The basic stance of a TFP therapist is therapeutic neutrality, that is, the stance of a neutral observer, not taking sides in relation to the patient's internal conflicts, and from this position fostering the patient's observation and understanding of her own conflicts. In essence, the therapist allies herself with the patient's capacity for self-observation. Encouragement by the therapist of the patient's capacity to articulate, observe, and reflect on her own conflicts is a major treatment goal, as reflection decreases reflex action and increases self-observation in times of stress.

Technical neutrality does not imply a passive or noncommittal attitude toward the patient on the part of the TFP therapist. Rather, while maintaining a neutral, nonjudgmental stance in relation to the patient's *conflicts*, the TFP therapist manifests active interest and curiosity in relation to the patient *as an individual*, seeking to understand the patient's experience in depth. By maintaining technical neutrality, the therapist protects a strategic position that allows for clarification and interpretation of dominant active transference paradigms that arise in the treatment.

Interpretive Process. The process of interpretation, as it is contained within the structure of the treatment frame, most defines TFP. Four discernable levels of intervention in the interpretive process have been described (Caligor et al., 2009; Clarkin et al., 2020). These levels take on individual detail and characteristics with each patient.

In the *first phase* of interpretation, the therapist works with the patient to elaborate the dominant object relation activated in the session,

highlighting the patient's experience of the other/therapist and the implicit self-perception embedded in that experience (e.g., the patient experiences himself as a victim in the hands of an aggressive, victimizing other/therapist). The therapist begins by inquiring into any vagueness, omission, or contradiction in the patient's depiction of self and others, while being careful not to call into question the patient's experience (i.e., staying within the patient's subjectivity while maintaining therapeutic neutrality). This mode of inquiry will typically lead to further patient affective reactions toward the therapist. The therapist next transforms these reactions into words, putting language to the patient's dominant experience of himself in relation to the therapist in the moment, along with the associated affect state ("I can see that you experience me as bullying you, while you feel diminished, helpless, and angry"). This process, when done well, results in the patient feeling understood by the therapist, while promoting affect containment and enhanced affect regulation in the moment.

The *second phase* in the interpretive process involves observing and identifying role reversals between self and object representations within the object relation dyad articulated in the first phase. These reversals are often enacted, most evident in the patient's behavior, while remaining dissociated from his dominant conscious experience. For example, while the patient perceives himself as a bullied victim in the hands of an aggressive, powerful other, the patient comes to aggressively attack and bully the therapist. In this process, the patient still sees himself in the familiar role of victim, while, without reflection and largely without awareness, behaves as victimizer and attacker. An alert therapist points out these role reversals, helping the patient to tolerate awareness of dissociated (typically aggressive) aspects of his behavior and experience, and then inviting him to reflect upon their possible meaning. ("I understand that you feel bullied by me. At the same time, I wonder if you can step back for a moment and consider that your behavior, raising your voice and berating me, might leave me feeling bullied as well.") By bringing the role reversal to the patient's attention, the therapist is inviting the patient to go beyond his immediate, concrete experience; the patient is supported in his capacity to step back and consider alternative perspectives, while forming cognitive connections between dimensions of behavior and experience that have been previously dissociated. This intervention promotes mentalization, inviting the patient to reflect on dissociated aspects of his behavior and internal experience, and to consider its impact on the experience of the other. In sum, tactfully calling attention to role reversals functions as a bid for reflection, inviting the patient to observe the way his mind works and

how it influences his behavior, and to explore this reality further within the therapeutic relationship.

In the *third phase* of interpretation, the therapist makes a connection between two contradictory object relations (typically idealized and persecutory experiences of self and other) that have been defensively dissociated. The therapist invites the patient to observe and reflect on the polarized and contradictory aspects of the experience of the therapist across time. ("I appreciate that you experience me right now as caring and nurturing. At the same time, I remember that at the beginning of the session I seemed a dangerous and aggressive bully.") This level of intervention calls attention to the discontinuous and polarized nature of the patient's experience in the transference, helping the patient to appreciate that his experience is internal and subjective, in contrast to material and concrete, and that subjective experience is constructed and symbolic. (For example, it is not possible that the therapist is, in fact, both the absolute best and the absolute worst as he has seemed at different moments.)

In the *fourth phase* of interpretation, the therapist provides hypotheses about the meaning of the patient's transference experience and the anxieties and motivations driving the patient's defenses. ("Perhaps you need at this moment to see me as perfectly nurturing and caring. It is as if you feel you must split off any possibility of negative feelings. As if you fear your positive feelings might be swamped by even a hint of the negative ones that were so powerful earlier in the session.")

It is important to emphasize that the process of interpretation begins at the cognitive-affective level that the individual patient brings to the treatment. Patients at different levels of personality organization bring different cognitive-affective capacities, coloring the quality of their self/other representations and their capacity for affective integration and regulation, setting limits on both the speed and the degree of change in the treatment. We have articulated the rating of the steps in the interpretative process for process research and to aid in reliable and consistent supervision (Keefe, Sowislo, & Clarkin, 2021).

THE PROGRESSION OF CHANGE

A close examination of multiple treatment cases, including published cases (e.g., Clarkin, Petrini, & Diamond, 2019), provides detailed information about the sequence of therapeutic challenges and patient progression of change in TFP-E. The progression of patient change depends upon the initial severity of the patients' pathology and other

patient pretreatment characteristics (Lenzenweger, Clarkin, Levy, Yeomans, & Kernberg, 2012).

Initial contract setting is tailored to the level of the patients' personality organization. The patient at a neurotic level of personality organization clearly sees herself/himself as having a problem and as in the role of seeking assistance from an expert. This clear role delineation is less salient with patients at a borderline level of organization. As the borderline level of severity increases, there is more need for the TFP therapist to clearly articulate the respective roles of patient and therapist as part of the treatment contract. The treatment contract also introduces the behavioral frame of the treatment, for example, being on time and ready to discuss what is on his/her mind for all sessions and accepting the need for volunteer or part-time work. Treatment will not begin until patient and therapist have formed a treatment contract. With borderline patients, internal acceptance of the contract and acceptance of what the therapist has to offer typically emerge only after careful exploration, involving clarification and confrontation, of the patient's attitude toward the necessary conditions of treatment.

Once the treatment begins, with the patient having accepted the conditions of treatment in the form of the treatment contract, the patient's conflicts begin to test the treatment frame in ways that invite therapist attention, clarification, and interpretation. The patient's response to the sequence of clarification, confrontation, and interpretation is a reflection of a test of the therapeutic alliance. As the severity of personality organization increases, it is more likely that the patient's initial response to therapist interventions is negative rebuttal, dismissal, or silent resistance. This phase can last for several months or more (see Clarkin et al., 2019), but in many cases even during this phase the patient begins to show hints of affectionate attachment to the therapist, with an increasing receptivity to therapist interventions.

From case data, we can identify patients who demonstrate symptomatic, often behavioral improvements (e.g., beginning to do volunteer work or working part time) but show no structural change (e.g., little change in negative perceptions of self and others, little improvement in friendships and intimate relations). The more complete change, including behavioral *and* structural change, seems to be related to pretreatment patient characteristics (e.g., lack of antisocial traits) and active, emotional engagement with the therapist and the therapy. Different domains of functioning change at different rates; patients at a borderline level of organization often improve in work/professional functioning before noticeable improvement in intimate relations (Lenzenweger et al., 2012).

HETEROGENEITY IN THE THERAPIST–PATIENT MATCH: THE ROLE OF PEER CONSULTATION

The psychotherapy research community has emphasized the need for a treatment manual that carefully describes the treatment, to ensure the treatment is actually being delivered, to make it possible to rate therapist adherence and competence, and to enable faithful replication of the treatment at different sites. These guidelines have influenced our research and our clinical work. In both, we utilize weekly peer consultation in a group format looking at videotaped therapy sessions. It is our observation that therapist adherence and competence in TFP is not a trait phenomenon but rather varies somewhat from case to case. A very talented and competent therapist can experience substantial difficulties in a particular case. This can happen in a number of ways, but, in general, it is related to the patients' behavior and the transference–countertransference dynamic, wherein the patient's transference has the effect of drawing the therapist into a collusion with the patient's dysfunctional attitudes and behavior. For example, one experienced therapist was drawn into the dismissive style of a borderline patient, and the therapist adopted that style in return. An overriding role of peer consultation is to alert the therapist to countertransference reactions that could be distorting the treatment. Often such countertransferences are at least to some degree outside of the therapist's awareness, requiring the "third eye" of the supervision group to be fully identified.

RECENT ADVANCES FROM THE EMPIRICAL EVIDENCE ON TFP

We are a group of medical college academics and practicing clinicians who are focused on examining our psychotherapy efforts with empirical methods. Our interests start with the pathology and treatment of the individual patient and expand to potential similarities between groups and subgroups of patients that become evident in our data. We have taken a systematic, stepwise approach to the empirical development of TFP (Kazdin, 2004), and briefly summarize here our advances in this area.

Articulation of a Principle-Driven Treatment Manual

A beginning step was the development of a principle-driven treatment manual (Clarkin et al., 1999) based on principles of intervention

used by senior clinicians working with seriously disturbed personality disorder patients as viewed on video recordings of actual sessions. Every clinician knows that no two patients are alike even given the same diagnosis, especially as the transference evolves over time. We have attempted to advance the art of psychotherapy by writing principle-based treatment manuals because every patient–therapist treatment episode is unique. It is the task of the therapist to apply the TFP-E principles to the unique and individual situation.

Our manuals have progressed from the first principle-driven manual for TFP and the treatment of borderline personality disorder, to the current manual describing the object relations treatment of patients with varying levels of disturbed personality organization (Caligor et al., 2018). This advance recognizes both the complexity and uniqueness of every patient and the flexible application of the key principles of TFP-E.

Treatment Development Study

The next step was a treatment development study funded by National Institute of Mental Health (Clarkin et al., 2001). Borderline patients were treated in 1 year of TFP. The treatment was well received, the dropout rate was low (19.1%), and none of the patients deteriorated or were adversely affected by the treatment. Compared to the year prior to treatment, patients experienced significantly fewer psychiatric hospitalizations, fewer days of inpatient hospitalization, and a reduction in the number of suicide attempts. These results suggested that TFP was effective in lowering health care costs incurred by emergency room visits and hospitalizations. The reduction in hospitalizations also provides room for the more normal development in the community for the patient.

Randomized Clinical Trial

Based on these encouraging results we conducted a randomized clinical trial in which TFP was compared to dialectical behavior therapy (DBT) and a dynamically oriented supportive treatment (Clarkin et al., 2007) delivered in individual treatment format over 1 year. Individual growth-curve analysis (Lenzenweger, Johnson, & Willett, 2004) was used to investigate change in symptoms and domains of functioning over time. All three treatments showed significant change across multiple domains after 1 year of treatment. However, only TFP and

DBT were associated with improvement in suicidality, and only TFP was significantly associated with improvement in impulsivity, irritability, and verbal and direct assault. Our impression from this first randomized controlled trial was that all three structured treatments were effective with borderline patients with significant structural pathology, but that TFP was unique in the reduction of aggression, possibly due to the close examination of aggression in the patients' transferences in treatment.

We utilized a subsample from this study to further explore the patient pretreatment predictors of change in three specific domains of functioning (aggression dyscontrol, global functioning, and anxiety/depression and impulsivity; Lenzenweger et al., 2012). Lower pretreatment levels of negative affect and aggression were associated with more rapid clinical improvement in aggressive dyscontrol. Higher pretreatment identity diffusion was associated with more rapid clinical improvement in global functioning. Lower initial levels of social potency were associated with more rapid improvement in anxiety/depression and impulsivity.

Randomized Controlled Trial in Different Setting

The next step in the empirical development of TFP was to evaluate the effectiveness of TFP in a cultural setting different from that of the original study in New York. Female borderline patients were randomized to 1 year of either TFP or treatment by community therapists experienced in the treatment of borderline patients in a two-site randomized controlled trial conducted in Europe (Doering et al., 2010). The patients treated with TFP showed many indications of superior improvement over the patients in the comparison treatment: reduction in borderline personality disorder criteria, reduction in suicide attempts, reduction in hospital treatment, and improvement in social and personality functioning. The results of this study suggested to us that the specific aspects of TFP as compared to treatment by local experts added to patient improvement. However, we were still not capturing and measuring the specific effects of the interpretive process in TFP.

With the TFP focus on understanding and improving internal representations of self and others, we hypothesized that TFP would result in changes in attachment organization and reflective functioning, which is a measure of the capacity to understand the behavior of oneself and others in terms of intentional mental states such as thoughts, feelings, and beliefs (Levy et al., 2006). Patients receiving TFP in both randomized controlled trials showed significant improvement as compared to

patients in the comparison treatments in terms of attachment status and reflective functioning (see Buchheim & Diamond, 2018). The positive influence of TFP on reflective functioning across studies suggested to us that the interpretive process was enhancing the patients' ability to reflect in a more benign way on self and others.

Neurocognitive Functioning

There is growing evidence that patients with borderline personality disorder manifest particular abnormalities in brain functioning associated with distorted thinking about self and others (higher connectivity between limbic and midline structures), hyperreactivity to negative stimuli (higher limbic activation; lower prefrontal activation), and impulsivity (lower ventral striatum activation; lower ventral striatum-frontal connectivity; Gunderson, Herpertz, Skodol, Torgersen, & Zanarini, 2018; Ruocco & Carcone, 2016; Skodol, 2018). In contrast to the growing data on brain functioning in individuals with borderline personality disorder, there is scarce data on the impact of treatment in modifying brain functioning in these individuals. The hypothesized mechanism of change in TFP is increased affect regulation achieved through mentalization, that is, the growing ability of the patient with borderline personality disorder to put momentary affect arousal, especially in social interactions, into a more benign and broader contextual understanding (Levy et al., 2006). We hypothesized that as the gradual analysis of the perception of self and others in TFP would modify the extreme cognitive/affective perceptions, these changes would be consistent with changes in neurocognitive functioning.

In a pilot study with $N=10$ borderline patients manifesting prominent affect dysregulation treated in TFP for 1 year, we found changes in brain functioning consistent with this hypothesis (Perez et al., 2016). Patients exhibited significant change in behavioral and psychological domains over the course of 1 year of TFP, including a reduction in affective lability, interpersonal sensitivity, and paranoia, and significant positive changes in work functioning.

In a comparison of pretreatment and posttreatment functional magnetic resonance imaging scans, patients with borderline personality disorder manifested relative increased activation in cognitive control regions (right anterior-dorsal anterior cingulate cortex and right dorsal-lateral prefrontal cortex) and relative decreased activation in the left inferior frontal gyrus and the left hippocampus. In addition, patients' self-reported improvements in cognitive control over the course of TFP correlated positively with left anterior-dorsal anterior cingulate cortex

activation, while improvements in self-reported affective lability correlated positively with left posterior medial orbitofrontal cortex/ventral striatum activation and correlated negatively with right amygdala/parahippocampal cortex activation. Finally, improvements in clinician-rated aggression over 1 year of TFP correlated positively with activation in the left inferior frontal gyrus. Taken together, these results suggest that treatment with TFP was associated with relative activation increases in emotional and cognitive control areas of the brain and relative activation decreases in areas of the brain associated with emotional reactivity and semantic-based memory retrieval.

SUMMARY AND CONCLUSIONS

The object relations model of personality functioning and pathology and related development of TFP have both contributed to and benefited from recent advances in the field of psychotherapy and psychotherapy research on the personality disorders. Object relations theory emphasizes both observable interpersonal behavior and the underlying affectively charged mental representations of self in relation to others that organize observed behavior. This view is quite consistent with the AMPD as defined by the *DSM-5* and assessment of self–other functioning along dimensional lines of severity of dysfunction. Our approach to treatment emphasizes the match of levels of personality pathology with the process of interpretation in the context of the treatment structure. This approach is both transdiagnostic in the sense of focusing on the core of personality dysfunction across the categories of personality disorder and also personalized for the individual patient. Whereas randomized clinical trials pit alternative treatments against each other, focusing on patients selected with specific personality disorder diagnoses, the approach of focusing on the relationship between patient and therapist as an essential ingredient in all psychotherapy outcomes (Norcross & Lambert, 2018) is quite consistent with the TFP approach to examining and understanding the therapeutic relationship as a major manifestation of the personality disordered patients' core difficulties in relating to others as a focus of needed change.

Since its initial articulation in 1999 (Clarkin et al., 1999) TFP has developed from an object relations treatment for patients with borderline personality disorder to a specialized approach to individuals' difficulties in self-functioning and functioning with others that is the heart of personality pathology (Caligor et al., 2018). A central theme in our clinical research efforts is to advance both the science (i.e., empirically based group effects) and the art of long-term intensive

treatment (i.e., focus on each individual patient) of patients with personality pathology.

Extensive clinical experience with the individual treatment format in delivering TFP has led to important modifications, including enhancing the treatment contract and structuring of the treatment, and placing emphasis on both the developing transference and the behavior of the patient in his/her current daily life. The principles of TFP have remained the same, but their application to a range of problems in self-functioning and functioning with others has expanded. As other articles in this special issue indicate, the principles of TFP are now extending to other treatment formats (e.g., group and family formats), and multiple treatment settings including inpatient, day hospital, and medical environments.

REFERENCES

- American Psychiatric Association. (2013). *Diagnostic and statistical manual of mental disorders (DSM-5)*. Washington, DC: Author.
- Barlow, D. H., Allen, L. B., & Choate, M. L. (2004). Toward a unified treatment for emotional disorders. *Behavior Therapy, 35*(2), 205–230.
- Buchheim, A., & Diamond, D. (2018). Attachment and borderline personality disorder. *Psychiatric Clinics, 41*(4), 651–668.
- Caligor, E., Diamond, D., Yeomans, F. E., & Kernberg, O. F. (2009). The interpretive process in the psychoanalytic psychotherapy of borderline personality pathology. *Journal of the American Psychoanalytic Association, 57*(2), 271–301.
- Caligor, E., Kernberg, O. F., Clarkin, J. F., & Yeomans, F. E. (2018). *Psychodynamic therapy for personality pathology: Treating self and interpersonal functioning*. Washington, DC: American Psychiatric Association.
- Clarkin, J. F., Caligor, E., & Sowislo, J. F. (in press). Transference-focused psychotherapy for levels of personality pathology severity. In H. Crisp & G. Gabbard (Eds.), *Gabbard's textbook of psychotherapeutic treatments* (2nd ed.). Washington, DC: American Psychiatric Association.
- Clarkin, J. F., Caligor, E., & Sowislo, J. F. (2020). An object relations model perspective on the alternative model for personality disorders (DSM-5). *Psychopathology, 53*(3), 134–141.
- Clarkin, J. F., Caligor, E., Stern, B. L., & Kernberg, O. F. (2016). The Structured Interview for Personality Organization—Revised (STIPO-R). Unpublished manuscript, Weill Medical College of Cornell University. Available from www.borderlinedisorders.com.
- Clarkin, J. F., Foelsch, P. A., Levy, K. N., Hull, J. W., Delaney, J. C., & Kernberg, O. F. (2001). The development of a psychodynamic treatment for patients with borderline personality disorder: A preliminary study of behavioral change. *Journal of Personality Disorders, 15*(6), 487–495.
- Clarkin, J. F., Levy, K. N., Lenzenweger, M. F., & Kernberg, O. F. (2007). Evaluating three treatments for borderline personality disorder: A multiwave study. *American Journal of Psychiatry, 164*(6), 922–928.

- Clarkin, J. F., Petrini, M., & Diamond, D. (2019). Complex depression: The treatment of major depression and severe personality pathology. *Journal of Clinical Psychology, 75*(5), 824–833.
- Clarkin, J. F., Yeomans, F. E., & Kernberg, O. F. (1999). *Psychotherapy for borderline personality*. New York: Wiley.
- DeRubeis, R. J., Cohen, Z. D., Forand, N. R., Fournier, J. C., Gelfand, L. A., & Lorenzo-Luaces, L. (2014). The Personalized Advantage Index: Translating research on prediction into individualized treatment recommendations. A demonstration. *PLoS ONE, 9*(1), e83875.
- Doering, S., Horz, S., Rentrop, M., Fischer-Kern, M., Schuster, P., Benecke, C., . . . Buchheim, P. (2010). Transference-focused psychotherapy v. treatment by community psychotherapists for borderline personality disorder: randomised controlled trial. *British Journal of Psychiatry, 196*(5), 389–395.
- Gunderson, J. G., Herpertz, S. C., Skodol, A. E., Torgersen, S., & Zanarini, M. C. (2018). Borderline personality disorder. *Nature Reviews Disease Primers, 4*(1), 1–20.
- Hallquist, M. N., & Pilkonis, P. A. (2012). Refining the phenotype of borderline personality disorder: Diagnostic criteria and beyond. *Personality disorders: Theory, Research, and Treatment, 3*(3), 228.
- Horz-Sagstetter, S., Caligor, E., Preti, E., Stern, B. L., De Panfilis, C., & Clarkin, J. F. (2018). Clinician-guided assessment of personality using the Structural Interview and the Structured Interview of Personality Organization (STIPO). *Journal of Personality assessment, 100*(1), 30–42.
- Kazdin, A. E. (2004). Evidence-based treatments: Challenges and priorities for practice and research. *Child and Adolescent Psychiatric Clinics, 13*(4), 923–940.
- Keefe, J. R., Kim, T. T., DeRubeis, R. J., Streiner, D. L., Links, P. S., & McMain, S. F. (2020). Treatment selection in borderline personality disorder between dialectical behavior therapy and psychodynamic psychiatric management. *Psychological Medicine, 1*–9.
- Keefe, J. R., Sowislo, J. F., & Clarkin, J. F. (2021). Transference-focused psychotherapy adherence rating scale. Unpublished manuscript, Weill Medical College of Cornell University.
- Kernberg, O. F. (1975). Transference and countertransference in the treatment of borderline patients. *Journal of the National Association of Private Psychiatric Hospitals*(2), 14–24.
- Kernberg, O. F. (1981). Structural interviewing. *Psychiatric Clinics, 4*(1), 169–195.
- Kernberg, O. F. (1984). *Severe personality disorders: Psychotherapeutic strategies*. New Haven, CT: Yale University Press.
- Lenzenweger, M. F., Clarkin, J. F., Levy, K. N., Yeomans, F. E., & Kernberg, O. F. (2012). Predicting domains and rates of change in borderline personality disorder. *Personality Disorders: Theory, Research, and Treatment, 3*(2), 185.
- Lenzenweger, M. F., Clarkin, J. F., Yeomans, F. E., Kernberg, O. F., & Levy, K. N. (2008). Refining the borderline personality disorder phenotype through finite mixture modeling: Implications for classification. *Journal of Personality Disorders, 22*(4), 313–331.
- Lenzenweger, M. F., Johnson, M. D., & Willett, J. B. (2004). Individual growth curve analysis illuminates stability and change in personality disorder features: The Longitudinal Study of Personality Disorders. *Archives of General Psychiatry, 61*(10), 1015–1024.

- Levy, K. N., Meehan, K. B., Kelly, K. M., Reynoso, J. S., Weber, M., Clarkin, J. F., & Kernberg, O. F. (2006). Change in attachment patterns and reflective function in a randomized control trial of transference-focused psychotherapy for borderline personality disorder. *Journal of Consulting and Clinical Psychology, 74*(6), 1027.
- Lingiardi, V., & McWilliams, N. (2017). *Psychodynamic diagnostic manual: PDM-2*. New York: Guilford Publications.
- Norcross, J. C., & Lambert, M. J. (2018). Psychotherapy relationships that work III. *Psychotherapy, 55*(4), 303.
- Norcross, J. C., & Wampold, B. E. (2018). A new therapy for each patient: Evidence-based relationships and responsiveness. *Journal of Clinical Psychology, 74*(11), 1889–1906.
- Panksepp, J., & Biven, L. (2012). *The archaeology of mind: Neuroevolutionary origins of human emotions (Norton series on interpersonal neurobiology)*. New York: WW Norton & Company.
- Perez, D. L., Vago, D. R., Pan, H., Root, J., Tuescher, O., Fuchs, B. H., . . . Clarkin, J. F. (2016). Frontolimbic neural circuit changes in emotional processing and inhibitory control associated with clinical improvement following transference-focused psychotherapy in borderline personality disorder. *Psychiatry and Clinical Neurosciences, 70*(1), 51–61.
- Ruocco, A. C., & Carcone, D. (2016). A neurobiological model of borderline personality disorder: Systematic and integrative review. *Harvard Review of Psychiatry, 24*(5), 311–329.
- Sharp, C., Wright, A. G., Fowler, J. C., Frueh, B. C., Allen, J. G., Oldham, J., & Clark, L. A. (2015). The structure of personality pathology: Both general ('g') and specific ('s') factors? *Journal of Abnormal Psychology, 124*(2), 387.
- Skodol, A. E. (2018). Impact of personality pathology on psychosocial functioning. *Current Opinion in Psychology, 21*, 33–38.
- Stern, B. L., Caligor, E., Clarkin, J. F., Critchfield, K. L., Horz, S., MacCornack, V., . . . Kernberg, O. F. (2010). Structured Interview of Personality Organization (STIPO): Preliminary psychometrics in a clinical sample. *Journal of Personality Assessment, 92*(1), 35–44.
- Wright, A. G., & Woods, W. C. (2020). Personalized models of psychopathology. *Annual Review of Clinical Psychology, 16*.
- Yun, R. J., Stern, B. L., Lenzenweger, M. F., & Tiersky, L. A. (2013). Refining personality disorder subtypes and classification using finite mixture modeling. *Personality Disorders: Theory, Research, and Treatment, 4*(2), 121.

John F. Clarkin, Ph.D.

Clinical Professor of Psychology in Psychiatry

Department of Psychiatry

New York Presbyterian Hospital—Weill Medical College of Cornell University

Westchester Division

21 Bloomingdale Road

White Plains, NY 10605

jclarkin@med.cornell.edu

214 CLARKIN, CALIGOR, AND SOWISLO

Eve Caligor, M.D.
Clinical Professor of Psychiatry
Clinical Professor of Psychology in Psychiatry
Columbia University Vagelos College of Physicians and Surgeons
122 East 42nd Street, Suite 3200
New York, NY 10168
eve.caligor@gmail.com

Julia F. Sowislo, Ph.D.
Instructor of Psychology in Psychiatry
Department of Psychiatry
New York Presbyterian Hospital—Weill Medical College of Cornell University
Westchester Division
21 Bloomingdale Road
White Plains, NY 10605
jfs2006@med.cornell.edu